SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cisplatin + etoposide with radiotherapy

Limited-stage small-cell lung cancer · combined chemotherapy and radiation · NCCP 00279 · version 8 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

Schedule
Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Kidney injury
- ☐ Hearing changes that may persist
- ☐ Numbness or tingling
- ☐ Nausea and vomiting
- ☐ Low blood counts / infection

Etoposide
CHEMOTHERAPY

- ☐ Low blood counts / serious infection
- ☐ Mouth soreness
- ☐ Hair loss
- ☐ Infusion allergy / low blood pressure

Discuss serious infection/sepsis and possible hospital admission, bleeding, infusion reactions, lung inflammation, painful swallowing/dehydration, fatigue, fertility, hair loss and individual material risks. Cisplatin kidney injury, hearing loss and neuropathy may persist; oral etoposide alternatives need separate handling and dosing explanation. Radiotherapy needs its own informed-consent discussion; chemotherapy consent does not replace it.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00279 v8.

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